The implementation of trauma-informed practice in a large American Indian community

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Introduction

The Ambit Network seeks to integrate evidence-based treatments and trauma-informed practice (TIP) into community systems of care for children exposed to trauma. Trauma-informed practice infuses and sustains trauma awareness, knowledge, and skills into organizational cultures, practices, and policy (NCTSN, n.d.). Over the next three years, the Ambit Network will be working with one of the largest American Indian (AI) tribes in Minnesota to accomplish a number of goals, including the development of a trauma-informed workforce across child-serving agencies. Such work is critical, as American Indian/Alaska Native (AI/AN) children are victims of child abuse and neglect more frequently than almost any other group of children in the United States. In 2012, the national rate of substantiated abuse and neglect was approximately 9.2 victims per 1,000 children, compared with 12.4 victims per 1,000 AI/AN children (US DHHS, 2013). Unfortunately, there exists a critical gap in the understanding of the facilitators and barriers of TIP implementation, particularly in underserved communities. Thus, we conducted two investigations of the implementation of TIP in the AI community of study. Study 1 evaluated the seven system-wide TIP trainings, assessing service providers' attitudes towards perceived change in daily practice, the cultural sensitivity of the trainings, etc. Study 2 investigated service providers' perspectives regarding factors that currently facilitate and prohibit TIP implementation within and across agencies. The study of TIP implementation is vitally important for both empirical and practical reasons, elucidating the effectiveness and sustainability of TIP in underserved settings.

Methods

Study 1: As part of the seven TIP trainings, all participants were asked to provide feedback through written evaluations. The 13-question evaluation assessed current TIP responsibilities (e.g., "Screen for trauma history in children"), the helpfulness of TIP training components, the required change to daily practice to implement TIP, and feelings towards making professional changes. Of the 320 participants, 239 (74.7%) completed an evaluation. See Table 1 for professional demographic data.

Study 2: A 30-question follow-up survey was developed using Qualtrics. Participants were contacted via email through snowballing methods. Participants were entered into a gift card drawing for reimbursement. Of the 73 respondents, 58 (79.5%) had attended an in-person TIP training, 6 (8.2%) had not attended a training, while 9 (12.3%) did not know whether or not they had attended a training. See Table 2 for professional demographic data.

Table 1. Professional demographics of training attendees for Study 1

Agency	n (%)
Education	93 (38.9)
Community Health	41 (17.2)
Substance Abuse	29 (12.1)
Indian Child Welfare	24 (10)
Mental Health	21 (8.8)
Law Enforcement	13 (5.4)
Tribal Court/Juvenile Justice	8 (3.3)
Other Agency	10 (4.2)
Total	239

Table 2. Professional demographics of survey respondents for Study 2

Agency	n (%)
Education	33 (45.2)
Community Health	17 (23.3)
Substance Abuse	5 (6.8)
Indian Child Welfare	5 (6.8)
Mental Health	8 (11.0)
Law Enforcement	1 (1.4)
Tribal Court/Juvenile Justice	1 (1.4)
Other Agency	3 (4.1)
Total	73

Data Analysis

To address study questions, a mixed-methods design was used to analyze quantitative and qualitative data. Quantitative analyses, including frequencies, chi-squared tests, and Mann-Whitney U tests were conducted using SPSS Version 22 for Mac OS X. Qualitative analyses were performed through initial and focused coding. Initial, line-by-line coding allowed for the separation of data into categories, while focused coding was used to sift through the remaining responses.

Results

Study 1

Quantitative analyses of the written evaluations from the TIP trainings addressed a number of study questions, including the required change to service providers' daily practice to implement TIP, feelings towards making professional TIP changes, and current TIP responsibilities. Evaluation results of interest are discussed below.

- Analyses indicated that service providers' perceived change to their daily practice to implement TIP significantly differed by agency, χ^2 (7, N = 224) = 180.11, p<.000. Mann-Whitney U tests revealed that the distribution of responses significantly differed between education and mental health, U(107), Z=-2.2, p<.05; and community health, U(126), Z=-2.5, p<.05. Approximately 73% of educators believed that they needed to make some or a lot of change to implement TIP, compared with about 48% of mental and community health providers.
- Analyses indicated that service providers' feelings towards making TIP changes significantly differed by agency, χ^2 (7, N = 218) = 180.11, p < .000. Mann-Whitney U tests revealed that the distribution of responses significantly differed between law enforcement and Indian Child Welfare, U(31), Z=-2.69, p<.01; education, U(99), Z=-2.3, p<.05; mental health, U(30), Z=-2.1, p<.05; and substance abuse, U(37), Z=-2.5, p<.05. Approximately 46% of law enforcement officers felt as though TIP was not going to work for their agency, compared with 4% of substance abuse, 5% of Indian Child Welfare, 12% of education, and 16% of mental health service providers.
- Distributions of current TIP responsibilities by service system agency are displayed in Table 3 and Figure 1.

Table 3. Percent of service providers endorsing trauma-informed practice responsibilities within current job roles, by agency

Trauma-Informed Practice Components	Indian Child Welfare	Law Enforcement	Tribal Courts/ Juvenile Justice	Education	Mental Health	Substance Abuse	Community Health	Other	Average Across Agencies
1. Have an understanding of child traumatic stress and trauma-informed practice	87.5	69.2	87.5	72.0	85.7	60.7	46.3	40.0	68.1
2. Consider how trauma is affecting a child's behavior	79.2	46.2	50.0	79.6	90.5	53.6	43.9	30.0	66.4
3. Work with other agencies to help children and families recover from trauma	83.3	53.8	37.5	41.9	85.7	50.0	53.7	30.0	52.9
4. Screen for trauma history in children	66.7	23.1	0.0	11.8	47.6	25.0	24.4	0.0	23.9
5. Have knowledge of a child and/or family's trauma history	83.3	46.2	87.5	48.4	81.0	39.3	53.7	10.0	54.2
6. Consider a child and/or family's trauma history in developing case plans	83.3	23.1	37.5	32.3	71.4	53.6	39.0	0.0	42.9
7. Know where I can send children for a trauma assessment	75.0	30.8	50.0	55.9	71.4	39.3	61.0	50.0	56.3
8.. Know where I can send children and/or families for trauma treatment	75.0	30.8	37.5	47.3	71.4	39.3	65.9	50.0	53.4

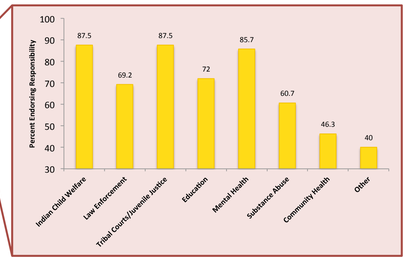


Figure 1. Percent of service providers endorsing the understanding of TIP as a responsibility within their job role, by agency

Study 2

In Study 2, qualitative analyses were conducted using initial and focused coding methodologies. Prompts of interest included each agency's TIP strengths, weaknesses, barriers to implementation, and barriers to the implementation of TIP across service systems. Table 4 displays the two most frequent themes that emerged from initial and focused coding for agencies with the greatest number of respondents.

Table 4. Qualitative analysis of the strengths, weaknesses, agency barriers, and system-wide barriers to implementing trauma-informed practice, by agency

Prompts	Education	n (%)	Community Health	n (%)	Mental Health	n (%)	Substance Abuse	n (%)
Agency's TIP Strengths	• Screening and assessment of students (and/or families)	6 (18.8)	• Knowledge of appropriate referrals	5 (50.0)	• Trainings for TIP and trauma-informed care	6 (42.9)	• TIP trainings for all staff	2 (66.7)
	• Access to counseling services	4 (12.5)	• Collaboration with other agencies	2 (20.0)	• Resourcefulness of staff	2 (14.3)	• Collaboration with other service providers	1 (33.3)
	Total	**32**	***Total***	**10**	***Total***	**14**	***Total***	**3**
Agency's TIP Weaknesses	• More trainings	9 (24.3)	• More assistance to families as a whole	2 (22.2)	• Provision of "Native-driven" or culturally sensitive practices	4 (36.4)	• Collaboration within and between agencies	2 (50)
	• More supportive services	6 (16.2)	• Lack of TIP understanding or support outside of agency	2 (22.2)	• Provision of community education and preventive services	2 (18.2)	• Understanding of TIP	2 (50)
	Total	**37**	***Total***	**9**	***Total***	**11**	***Total***	**4**
Agency's Barriers to Implementing TIP	• Lack of communication between administration and staff	5 (20)	• The time TIP takes	2 (40.0)	• Short-staffed due to high rates of turnover	6 (60.0)	• Lack of proper planning	1 (50)
	• Lack of appropriate resources (e.g., counseling staff, funding)	4 (16)	• Lack of skills training	1 (20.0)	• Lack of resources or funding	2 (20.0)	• Lack of understanding or support within agency	1 (50)
	Total	**25**	***Total***	**5**	***Total***	**10**	***Total***	**2**
Barriers to Implementing TIP Across Systems	• Political tension within and and among agencies	3 (13.0)	• Confidentiality vs. collaboration with other agencies	3 (60.0)	• Lack of TIP understanding or support outside of agency	3 (37.5)	• Lack of understanding or support outside of agency	1 (33.3)
	• Confidentiality or data protection	3 (13.0)	• n/a		• Systems not willing to collaborate	3 (37.5)	• Systems not willing to collaborate	1 (33.3)
	Total	**23**	***Total***	**5**	***Total***	**8**	***Total***	**3**

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Conclusions

Trauma-informed practice (TIP) infuses and sustains trauma awareness, knowledge, and skills into organizational cultures, practices, and policies (NCTSN, n.d.). The Ambit Network of Minnesota is implementing TIP in one of the largest AI communities in the state, and is evaluating the implementation process through the assessment of system-wide TIP trainings, as well as factors that facilitate and prohibit implementation within and across agencies. Specifically, Study 1 evaluated seven TIP trainings, surveying service providers' attitudes towards perceived change in daily practice, feelings toward making professional TIP changes, and current TIP responsibilities. In contrast, Study 2 qualitatively identified each agency's TIP strengths, weaknesses, and barriers in the implementation of TIP on the reservation.

Quantitative analyses from Study 1 indicated that educators, in particular, believe that a significant amount of change in their daily practice is needed to implement TIP as discussed in the trainings, as compared with mental health and substance abuse providers. Additionally, significantly more law enforcement officers feel as though TIP is not going to work for their agency, compared with Indian Child Welfare, education, mental health, and substance abuse providers. Qualitative analyses from Study 2 suggested that the barriers associated with implementing TIP vastly differ by agency, ranging from the lack of communication between administrators and staff members, to high rates of staff turnover and the lack of appropriate resources (e.g., funding, time).

There is an exceptional dearth of empirical research studying the implementation of TIP, particularly in underserved areas like as AI communities. Aaron and Palinkas (2007) suggest that the implementation of evidence-based practices is often a complex process that is "fraught with unanticipated events, conflicts, and resolutions" (p. 411). In fact, the nature of the implementation process, the dilemmas encountered, and the solutions devised are dependent on the perspectives and experiences of a given group (Aaron & Palinkas, 2007). Although a number of common process factors have emerged in implementation science (e.g., training, communication, and supervision) that take place across service systems and types of services, an AI reservation presents unique challenges and opportunities to study TIP implementation (BigFoot & Schmidt, 2010). For example, the relationship among tribal, state, and national governments is often very complicated and usually not completely (or at all) understood by external researchers (BigFoot & Schmidt, 2010). Nevertheless, the study of TIP implementation in AI communities is of vital importance in determining both the effectiveness and sustainability of such an approach in underserved settings.

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Further Information

You can find more information about Minnesota's Ambit Network at http://www.cehd.umn.edu/fsos/projects/ambit/. Please contact the lead author with questions at doni0009@umn.edu.